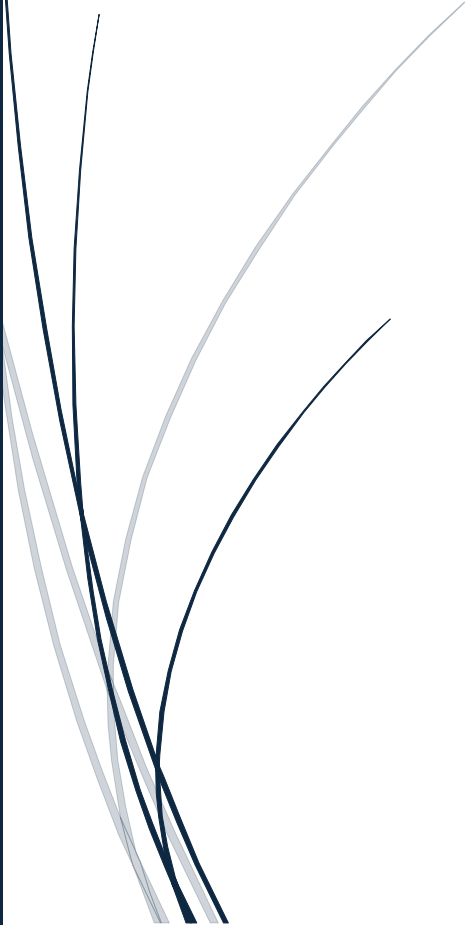




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Samaritans Crisis Trends Analysis Report

Dataset:- ↓ Samaritans Crisis Stats



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Executive Summary

This report analyses official suicide statistics from the ↓ Samaritans Crisis Stats dataset. The analysis focuses on long-term crisis trends, gender differences and regional variation. The key objective is to understand suicide patterns over time and identify groups and regions with higher levels of risk.

The analysis shows that suicide deaths in England and Wales reached 6,190 in 2024, with an age-standardised suicide rate of 11.4 per 100,000 population. Male suicide deaths remained substantially higher than female deaths. Regional analysis also showed clear variation, with the North East recording the highest suicide rate in 2024.

Dataset Overview

The dataset contains official suicide statistics for England and Wales. Two main tables were used:

Table	Purpose
Table 1	Suicide deaths and rates by sex, England and Wales, 1981–2024
Table 4	Suicide deaths and rates by English region, 1981–2024

The main variables used in the analysis were:

Variable	Description
Year	Year of death registration
Persons_Deaths	Total suicide deaths
Persons_Rate	Age-standardised suicide rate per 100,000
Male_Deaths	Male suicide deaths
Male_Rate	Male suicide rate per 100,000
Female_Deaths	Female suicide deaths
Female_Rate	Female suicide rate per 100,000
Region	English region
Rate_2024	Regional suicide rate in 2024

Methodology

The analysis was completed using Python. The main steps included:

1. Loading the Excel workbook.
2. Extracting Table 1 for long-term trend analysis.
3. Cleaning column names and converting values into numeric format.
4. Performing exploratory data analysis.
5. Creating suicide trend charts.
6. Extracting Table 4 for regional analysis.
7. Creating regional variation charts.
8. Interpreting findings and preparing commentary.

Exploratory Data Analysis

The dataset required cleaning because the original workbook contained titles, notes and multi-level table headers. The useful data started after the metadata rows.

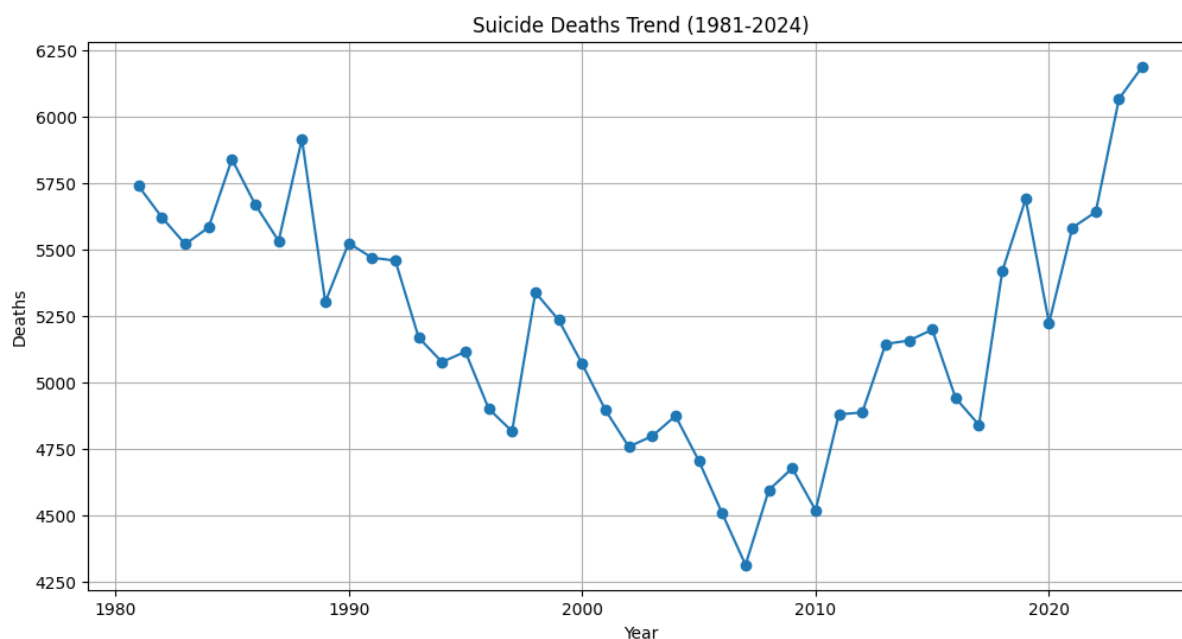
The cleaned Table 1 included annual suicide deaths and suicide rates for persons, males and females from 1981 to 2024. Missing values and non-numeric values were handled using numeric conversion and row filtering.

The cleaned Table 4 included 2024 regional suicide deaths and rates for the nine English regions.

Crisis Trend Analysis

The long-term suicide trend shows fluctuation over time, with recent years showing a rise in suicide deaths. In 2024, England and Wales recorded 6,190 suicide deaths.

This indicates that suicide remains a significant public-health and crisis-support concern. The 2024 figure represents a high point in the recent series and suggests continued demand for prevention, intervention and crisis-support services.



Suicide Rate Analysis

The age-standardised suicide rate in 2024 was 11.4 per 100,000 population. This rate provides a better comparison over time because it adjusts for changes in population age structure.

The rate trend shows that suicide risk has not remained constant. Some earlier years recorded lower rates, while recent increases suggest renewed public-health pressure.

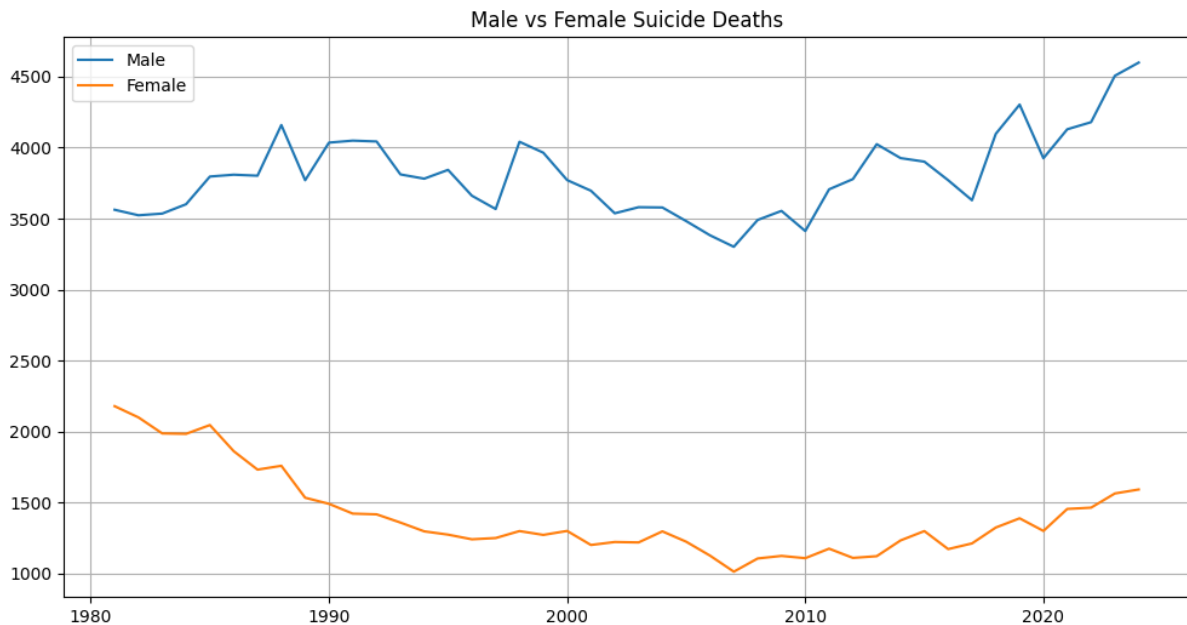


Gender Analysis

The analysis shows a strong gender gap in suicide deaths.

In 2024:

Sex	Deaths
Male	4,599
Female	1,591

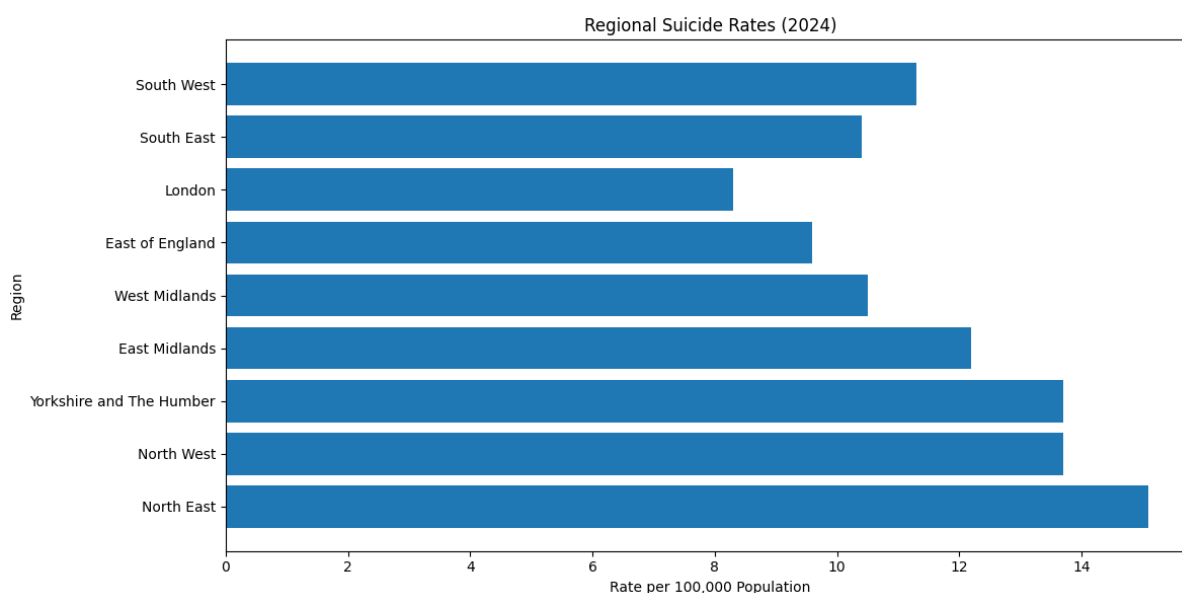


Male suicide deaths were almost three times higher than female deaths. This pattern highlights the need for male-focused suicide prevention strategies, including early intervention, crisis support, workplace mental-health programmes and community outreach.

Regional Variation Analysis

Regional variation was clearly visible in the 2024 data.

Region	Suicide Rate per 100,000
North East	15.1
North West	13.7
Yorkshire and The Humber	13.7
East Midlands	12.2
South West	11.3
West Midlands	10.5
South East	10.4
East of England	9.6
London	8.3



The North East had the highest suicide rate in 2024, while London had the lowest. This suggests that suicide prevention planning should consider regional differences rather than relying only on national-level strategies.

Key Findings

1. Suicide deaths reached 6,190 in 2024.
2. The 2024 suicide rate was 11.4 per 100,000 population.
3. Male suicide deaths were significantly higher than female deaths.
4. The North East recorded the highest regional suicide rate in 2024.
5. London recorded the lowest regional suicide rate.
6. The findings show the importance of targeted crisis-prevention strategies.

Public Health Interpretation

The analysis suggests that suicide remains a serious and ongoing mental-health challenge in England and Wales. The high number of deaths in 2024 indicates continued demand for crisis-support services. The strong male risk pattern suggests that prevention programmes should prioritise men, especially in high-risk regions.

Regional differences also show that mental-health support should be geographically targeted. Areas such as the North East, North West and Yorkshire

and The Humber may require additional crisis-support capacity, awareness campaigns and early intervention services.

Limitations

This dataset provides suicide registrations, not direct Samaritans call-volume data. Therefore, the analysis uses suicide statistics as a proxy for crisis trends. The dataset also does not contain monthly call-volume data, so seasonal patterns could not be directly measured from this workbook.

However, the dataset is still suitable for crisis-trend analysis because suicide statistics are a critical public-health indicator linked to crisis-support demand.

Recommendations

1. Strengthen suicide prevention programmes in high-rate regions.
2. Prioritise male mental-health outreach and crisis support.
3. Continue long-term monitoring of suicide deaths and rates.
4. Combine suicide statistics with helpline usage data when available.
5. Develop regional dashboards to track crisis trends and support planning.

Conclusion

The analysis confirms that suicide remains a major public-health concern in England and Wales. In 2024, suicide deaths reached 6,190, with men accounting for the majority of deaths. Regional variation was also evident, with the North East recording the highest rate.

Overall, the findings support the need for targeted suicide prevention, regional planning and continued investment in crisis-support services.